



Swansea University
Prifysgol Abertawe



GRADUATE ENTRY MEDICINE | CLINICAL PLACEMENT NEWSLETTER

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Welcome to the Clinical Placement Newsletter

In this issue, as ever you'll find news and updates on clinical placements. You can also find information about our great team of Honorary Senior Lecturers working across Wales. Students are occasionally somewhat unclear on who the HSLs are in their various secondary care placements, so, we thought we could address that gap. We have some great tips for getting the most out of placement from clinical fellows and students. Final year student Charlie Burton offers his thoughts on the hospital canteens across Wales, and we share Year 1 student Tiffany Edmond's fantastic artistic reflection on her placement experience at Wlthybush.



Don't forget you can access this and an archive of all newsletters here: <https://www.swanseaclinicalnewsletter.com/>

We appreciate your feedback! Please get in touch if you have any feedback or comments, or if you would like to contribute a reflection: 509248@swansea.ac.uk



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Updates from the Clinical Placements Team



Swansea University Medical School Clinical Placements Team (from left to right): Dr Geraint Morris, Lead for Clinical Apprenticeships and Assistantships; Professor Sujoy Banerjee, Lead for Secondary Care Clinical Placements; Dr Lisa Williams, Lead for Specialty Attachments; Dr James Kerrigan, Deputy Community Based Learning (CBL) Lead; Dr Llinos Roberts, CBL Lead. Dr Jonathan Salisbury (not pictured) is also on the CBL team.

The function of the Placements team is to work with clinical colleagues to ensure that all clinical placements are valuable and enjoyable learning experiences for students. We liaise closely with clinical partners across Wales in both Primary and Secondary care settings. We also work well together in a cohesive team! Here are our contact details:

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Clinical Apprenticeships (CAs)

Last year, **primary care placements were included in clinical apprenticeships** in year 1 and 2 for the first time, and, having now bedded in, these placements have drawn very positive student feedback. 91% of students on primary care clinical apprenticeships rated the placement "good or very good" (the equivalent for secondary care was 90%). Secondary care placements also went well. The highlight of the feedback from CA2 this year was the Pathology and Clinical Diagnostics (PCD) rotation, which was very well received by students. Radiology and phlebotomy experiences were particularly highly rated.

A small number of concerns were submitted in the student feedback from secondary care placements in relation to engagement of clinical teams and these have been communicated back to relevant Health Boards, units and clinicians. Actions have been taken and the situation will continue to be monitored by the placements team. Some conflicting advice given to students at the induction talks during the first day of their placements has been ironed out; all

Updates from the Clinical Placements Team

undergraduate teams accepting students now receive a slide with up-to-date information on required assessments in placements for use in induction talks. Cross-site working caused some unease last year but this was much less of an issue in CA2 this year.

This year, following on from a piloting exercise last year, we have included **2 new placement opportunities for the PCD rotation**. A small number of students can now experience spending 2.5 days in the Swansea Bay UHB Joint Clinical Research Facility and with the Cancer Research team or 2.5 days in the Nuclear Medicine unit at Singleton hospital. Feedback from both experiences has been very positive – though as they are new experiences, we are open to receiving suggestions for improvement.

We have not experienced the challenges of GP tutors not being fully familiar with marking written clerkings that we had last year (see Edition 7 of the Newsletter) – GPs are now more aware of students' assessment requirements this year following a lot of supporting work from Drs Kerrigan and Roberts. There remain some challenges with accommodation arrangements in a minority of primary care placements; our team is actively working on this, though funding constraints are a particular challenge. **We would like to thank students who have brought all their concerns to us for doing so** – without your valuable and constructive feedback it is difficult to make improvements!

A reminder about on-calls and assessments in CAs:

On calls in Clinical Apprenticeships:

In Year 1, no out of hours on call work is required over and above the one mandatory “out of hours” LOCS.

In Year 2 - one out of hours “on call” session is required during one of your clinical apprenticeships (in addition to the “out of hours LOCS”). This on call session should be done in a hospital-based specialty in a “non-rotational” placement ie not in the PCD rotation. The on-call session requires sign-off from the accompanying clinician on GEM Portal.

Assessments in Clinical Apprenticeships:

Year 1

1 “Written Clerking”, 1 “ROCS” - submit Written Clerking by end of 2nd week of placement in order to receive feedback before the end of placement (absolute deadline is end of CA1 placement), ROCS before end of placement.

Year 2

2 ROCS, 1 Written Clerking, 1 CBD

Complete 1 ROCS and 1 Written Clerking in first “non-rotational” (ie not the PCD rotation) placement - submit Written Clerking by end of 2nd week of placement, ROCS before end of placement

Complete 1 ROCS and 1 CBD in the second non-rotational placement

Updates from the Clinical Placements Team

Specialty Attachments

Changes to the placements for this academic year have been well received by students particularly in relation to **streamlined Learning Outcomes & increased Self-Directed Learning time**. The **increased number of teaching fellows** has also improved student experience. Students have fed back that the wide variety of clinical experiences is valuable.

There are still **challenges with cross-site working**. Taxis are provided for travel across site by the undergraduate teams, where public transport is inadequate. Cancellation of sessions at short notice can still be problematic and this is confounded by the extreme clinical pressures in the NHS.

Additional placements in Hywel Dda for **Mental Health & Women's Health** have gone very well with teaching fellow support on site.

Feedback – Completing the Loop

Opportunities for letting students know about our actions following their feedback have increased of late. These actions are communicated at Council meetings and updates are posted on Canvas for generic feedback following placements. This is in addition to feedback discussed in our Newsletter and with placement reps.

Junior Assistantships (JAs)

Another change in academic year 2024/25 was the introduction of **6-week long JAs** which seems to have been a positive development: students really appreciate the opportunity to get fully immersed in clinical teams and get going with the development of their foundation doctor skills.

Undergraduate management teams have also been providing excellent support at all sites, and this features highly in the end of placement student feedback.

Senior Student Assistantships (SSAs)

These will continue across Wales as with all the Welsh Medical Schools. If you are allocated a Foundation Year 1 (FY1) post in Wales, you will be in the SSA with the current FY1 in post, as this will prepare you to start as a foundation doctor. If you are allocated outside Wales then you will be allocated an SSA post that hasn't got a Welsh graduate and this could be anywhere in Wales. Until the deanery allocates to posts it is not possible to identify where all students will go for SSA. An All-Wales SSA handbook will be available before the SSA starts and will explain in detail the purpose and requirements needed.

Feedback from last year's SSA were very good from students, and the clinicians felt our students were prepared for practice, which is fantastic.

Teaching fellows

Teaching fellows have featured positively in the feedback from all placements and they seem to have made a significant impact on the quality of students' experiences. Teaching fellows from across Wales and across the 3 Medical Schools in Wales are currently planning to get together to form a group primarily for sharing good practice.

Supporting and Informing Clinical Teams

As a placement team we have been very busy meeting up with clinical teams in both primary care and secondary care settings. These meetings have taken the form of workshops, Faculty Development meetings, site visits by senior members of the Medical School, attendance at departmental audit meetings as well as individual conversations and correspondence. These meetings have all been helpful in optimising placements despite sometimes overwhelming clinical pressures faced by clinicians. We have more meetings planned this term including a site visit to our West Wales and North Wales units.

Updates from the Clinical Placements Team

Common Rooms

In Morriston hospital, with the large numbers of students present there on placement, there have been issues with **overcrowding in the common room**. The undergraduate team at Morriston will arrange for further rooms to be available for students if there are large numbers present – students are welcome to call in with the team in the Education office at Morriston should this be a problem.

Lack of microwave ovens has also been an issue with queues forming regularly. **Additional microwave ovens are on order!**

Compensatory Rest

There was some confusion in JA1 about students' entitlement to time off following time spent working out of hours at the beginning of this year, but this was clarified with undergraduate teams and students involved at the time. Compensatory rest time is explained in the GEM placements Handbooks, but essentially, in all placements, arrangements for compensatory rest following out of hours work now mirrors current EWTD/BMA Resident doctors' guidance.

Please remember...

- **Attendance** on clinical placements is very important and the Medical School takes attendance seriously. If you have any issues with attendance, please remember to discuss this with your undergraduate office team. This is only because so much learning takes place on clinical placements.
- **Punctuality** is important – if you genuinely cannot make a session due to unforeseen circumstances, please remember to let your team know and to politely apologise if you walk into a session late. This makes for good professional working relationships.

- Since the last Newsletter, we have had no recurrence of previous concerns about **student dress** from healthcare staff members. Please keep up the good work and stick to the Medical School advice on dress as well as the NHS dress code!
- A reminder – please **don't use mobile phones/tablets** in teaching sessions without prior permission and/or clarification with the tutor.

And Finally...

The feedback we get from clinical teams is extremely complimentary – the large majority of students are conducting themselves with a high standard of professionalism and this is being noted and appreciated by clinical colleagues.

Overall, placements have gone very well this academic year and **the placement team would like to thank all undergraduate teams, GP practices and clinical partners in all the placement sites** for their hard work in making these placements as useful and enjoyable as possible for all students. **We would also like to thank all our students** who have represented Swansea Medical School so well in their placements. Please remember that our doors are always open to any concerns or difficulties that you may have!

Honorary Senior Lecturers

***Honorary Senior Lecturers (HSLs)** are senior clinicians based in each hospital who represent the interests of Medical Students and Medical Schools in that hospital or hospitals. They oversee placement experiences in each unit and address any issues students have whilst on placement. It's useful and important for students to know who to approach if they have any issues, so here they are!*



**Mr Amol Pandit, HSL at Neath Port Talbot Hospital,
Swansea Bay University Health Board**

I am Amol Pandit, the Honorary Senior lecturer at Neath Port Talbot Hospital.

Along with 2 tutors that assist me, and Jill Miles the Education Manager, I oversee the student placements at NPTH. These include Clinical Attachments, Speciality Attachments and Junior Assistant placements.

Neath is the smallest of the SBUHB Hospitals but a very good one for student placements. There are a variety of opportunities available and rapidly expanding too.

We have a dedicated simulation training room, rest facilities with lockers, a good library and a very good cafeteria.

My other roles are:

BMA Welsh Council Member & Co-Chair BMA BAME Forum Wales

BMA Welsh Council Equality Champion

Co-Chair BAME Forum SBUHB

Joint Appraisal Lead, Morriston

Honorary Senior Clinical Lecturer Swansea University Medical School & HSL NPTH

Chair MSAC, Morriston

Member – Joint Local Negotiating Committee

Honorary Senior Lecturers

The Singleton Hospital HSL role is shared between Dr Lisa Williams and Dr Geraint Morris



Hi, my name is Lisa Williams and, along with Dr Geraint Morris, I am one of the HSLs in Singleton Hospital. I am a Gastroenterologist and Physician based at Singleton but also cover the Gastroenterology wards and medical intake at Morriston, so can often be found there too. Some of you will already have met me, as I lead one of the Learning weeks in year 1 and am Year 4 Lead. I also have a role in the Clinical Placement team and look after the Year 3 Speciality Attachment placements. I am easy to find through the undergraduate teams in Swansea Bay and happy to meet up with any students coming through the hospitals. I usually see you at mid placement catch ups but please feel free to get in touch if you have any concerns whilst at Singleton. I am also a Royal College Tutor for medicine in Swansea Bay so happy to chat through career options and postgraduate opportunities if you wish.

Singleton is a lovely hospital with clinical teams and undergraduate staff who are happy to support you. I am sure you will all enjoy.

Lisa x

Lisa.williams5@wales.nhs.uk

Many of you will already have come across me as I am an ICM tutor at the Medical School and run a Paediatric Cardiology clinic at Singleton which is one of the LOCS options. I am also Lead for Clinical Apprenticeships and Junior Assistantships at the Medical School. I trained mainly in Wales but also at Bristol, and since 2001 have been a Consultant Paediatrician/Neonatologist based primarily on the Neonatal Intensive Care Unit at Singleton. I have had many roles in both undergraduate and postgraduate education and am passionate about students having a top-quality experience in their placements both in Singleton hospital and across Wales. Along with Lisa, we offer an open door policy for students who wish to speak to us about anything placement-related, and can be contacted via email or via any of the undergraduate offices in Swansea Bay.

Geraint.morris@wales.nhs.uk



Honorary Senior Lecturers



Dr Angharad Piette, HSL for Psychiatry, Swansea Bay University Health Board

I am the Honorary Senior Lecturer for mental health placements in Swansea Bay Health board. In this role I help to co-ordinate placements for students from years 1-4 and am here to support and trouble shoot if there are any difficulties during these placements.

In my clinical role I am the consultant psychiatrist for Uned Gobaith mother and baby unit at Tonna hospital. I am passionate about perinatal psychiatry and providing good mental health care to all our patients. We love having medical students with our team and have learnt a lot from the interesting questions and discussions with our students.

Dr Rangaswamy Mothukuri, HSL at Morriston Hospital

I am a consultant in Emergency Medicine and Intensive Care Medicine in a dual role with a 50:50 job commitment in each speciality. I have been a dual consultant for 10 years.

I am the Honorary Senior Lecturer (HSL) for Morriston Hospital and I supervise the medical student's placements in Morriston Hospital in association with Swansea University and Medical School. I also do regular SIM training for the third-year medical students and have the week lead role for the first-year medical students.

I have an interest in sepsis and lead the SeGMED group (Sepsis Group Morriston Emergency Department). I am the Clinical Lead for Sepsis for Swansea Bay Health Board. As part of my dual role, I supervise the critical care and resuscitation related matters in our ED

I am interested in education and training. I supervise the ACCS-EM trainees at Morriston ED and supervise IMT trainees during their ITU placement.



Honorary Senior Lecturers



Dr Ben Davies, HSL Prince Philip Hospital, Hywel Dda University Health Board

I am a Consultant Physician and Gastroenterologist; I trained in Imperial College London and then made my way back west along the M4 completing Foundation and Medical in Bristol and then taking up specialist training in Wales.

I took time out of training to undertake a Postgraduate Diploma in medical education in Swansea University whilst being a teaching fellow in Prince Phillip Hospital (PPH) alongside being an honorary clinical lecturer with The GEM programme in particular with ICM.

My priority as Honorary senior lecturer for Prince Phillip Hospital is to maximise learning opportunities for students attending placement here by providing structure to the working week, utilising other specialties not always easily experienced in secondary care placements and include more time for students in the acute medical take. I feel this will allow us to continue to increase student numbers in PPH while not sacrificing the high standards of medical education we provide.

I live with my wife and three small children in Mumbles and love nothing more than getting out on the beautiful Gower cliffs with our dog Bryn.

Dr Kausik Khan, HSL at Bronglais Hospital, Hywel Dda University Health Board

Overseas paediatric postgraduate from India, Completed CCT in UK and developed special interest in paediatrics gastroenterology and allergy. Teaching is a fun time beyond clinical work. When not working, love eating good food and listen to music of different genre ranging from both Western and Indian classical to pop or R&B or country and of course bollywood but not a fan of hard rock. 😊

Honorary Senior Lecturers



Dr Chris James, Withybush Hospital HSL, Hywel Dda University Health Board

I am a consultant physician working in Hywel Dda University Health Board based at Withybush Hospital in Pembrokeshire. I have worked as a general physician with an interest in elderly medicine since my appointment in 1994. I run a Parkinson's clinic along with a Parkinson's Nurse specialist. I have been on the general medical on call rota from 1994-2022.

I have been a lead locally in Undergraduate education since 1998. I am an Honorary Senior Lecturer at the School of Medicine at Cardiff University and Honorary Clinical Senior Lecturer in the

College of Medicine, Swansea University.

Between 2010-15 I was a case manager in the Medic Support Programme which is a confidential service that offers assessment and remediation for medical students and doctors in Wales who are experiencing difficulties in non-clinical aspects of their performance.

I have been a Fellow of the Academy of medical educators since 2015.

Since the end of 2016 I have been lead for mentoring and medical recruitment within the Health Board. I have established a peer-mentoring programme for consultants of all specialties and a similar programme for SAS doctors across the Health Boards four hospitals.

Dr Richard Pugh, HSL at Glan Clwyd Hospital, Betsi Cadwalladr University Health Board

I'm Undergraduate Lead at Glan Clwyd Hospital, and Anaesthetist and ICU consultant by background.



Top Tips for Medical Students on Placement

Dr Scott New, Clinical Teaching Fellow in Medicine at Glangwili General Hospital, offers his top tips for making the most out of hospital placements:

Hospital Placements: Advice to my former self

Heading into hospital placements can feel exciting, nerve-wracking, and maybe even a little overwhelming—but don't worry, we've got you covered. Here are some friendly tips to help you thrive in the wards, theatres, and clinics while making the most of this incredible learning opportunity.



1. Show Up and Shine

First impressions matter, so arrive on time and come prepared. A little reliability goes a long way with your team, and punctuality shows you're taking your placement seriously. Find out what time the ward round starts as each Consultant likes to start at different times!

2. Dress to Impress (and Stay Comfortable)

Stick to the dress code—clean scrubs or professional attire, comfy shoes (your feet will thank you), and your ID badge proudly displayed.

3. Know Your Role

You're there to learn, so focus on observing, asking questions, and helping out within your abilities. Don't stress about knowing *everything*—this is your time to learn. Also do not feel like you are 'in the way' or 'annoying', these are inherent parts of your role, so own them.

4. Be Keen but Not Pushy

Show interest, volunteer when appropriate, and ask if you can assist—but always know your professional boundaries.

5. Do Your Homework

A quick review of the day's specialty or procedures (e.g., key anatomy or common conditions) can make a huge difference. It's a confidence booster and impresses the team.

6. Introduce Yourself

Start each day by introducing yourself to the team. Something like, "Hi, I'm [Your Name], a third-year medical student" works perfectly. Being approachable makes it easier for others to involve you.

Top Tips for Placement

7. Connect with Patients

Patients are your best teachers! Practice empathy and take time to really listen to their stories. This is the perfect chance to hone your history-taking and examination skills without the pressure of an OSCE examiner looking over your shoulder...

8. Confidentiality is Key

Keep patient information private—always. Throw away those patient lists in a confidential bin at the end of the day. Confidentiality is non-negotiable and it is your responsibility.

9. Get Hands-On (Safely)

If there's a chance to practice skills like taking blood or suturing, go for it—but only with supervision and after confirming you're ready. Don't be afraid to ask for guidance!

10. Stay Flexible

Hospitals can be unpredictable. Schedules shift, emergencies pop up—just go with the flow and see it as part of the learning adventure. Out of hours will often yield better experiences and opportunities as there are less students and less doctors, but the patient numbers stay the same!

11. Learn from Everyone

Doctors, nurses, physios, healthcare assistants—everyone has wisdom to share. Respect and value the entire team's role in patient care.

12. Be Honest About What You Don't Know

No one expects you to be perfect. If you're unsure about something, ask. It's much better than guessing—and it shows you're committed to learning.

13. Reflect on Your Day

Take a few minutes each evening to think about what went well, what you learned, and what you'd like to improve. Keeping a logbook of cases can help, too.

14. Seek Feedback

Ask for feedback from your supervisors and act on it. It's one of the fastest ways to improve—and it shows you're serious about becoming the best doctor you can be.

15. Take Care of Yourself

Placements can be intense, so remember to eat, stay hydrated, and sneak in a break when you can. Good sleep and looking after your mental health will keep you on top of your game. You have a whole career ahead of you to stress out.

Placements are an amazing time to build confidence, refine your skills, and learn from real-life scenarios. Enjoy the journey, embrace the challenges, and don't forget—you're not alone. Your supervisors, peers, and patients are all part of the learning process. As a Clinical Teaching Fellow, it is my role to support you in your placement here. There are an increasing amount of Clinical Teaching Fellows working in hospitals, so don't be afraid to reach out.

Top Tips for Placement

Lebba Manokaran, Year 1 GEM student, provides advice on starting clinical placements in primary care:

Having had very little hands-on clinical experience, starting my first clinical placement was very daunting. For the first couple of days, I observed doctors conduct concise patient histories, examine and investigate differentials in a GP. Every 15 minutes, there was a new patient – a new health problem and trying to find the missing jigsaw piece in a puzzle. I understood how important it was to listen to the patient's story and ensure that their concerns were being addressed. Regardless of how enthusiastic I may sound, settling into a new clinical environment can be overwhelming for students at first, especially for those from non-science backgrounds. I hope that by sharing my tips and experiences – I can shed some light onto the world of primary care.



To make the most out of CA1 in primary care, I've listed a few items you may want to bring with you and other preparation tips, that I found useful:

List of items:

- ✓ Student Name Badge (from the Education Centre)
- ✓ A small pocket-size notebook – helped me a lot! Especially with writing out medications that I was unfamiliar with. Sometimes, certain supplements were not on the system because it was prescribed privately. It made things easier when I was reporting back to the GP.
- ✓ Stethoscope – use the opportunity to examine patients
- ✓ Laptop/Tablet – easier to fill out assessments on the GEM portal

Top Tips for Placement

How should you prepare prior to CA1?

- ✓ Try to book and get your clinical skills signed off from the Clinical Skills Labs if you can. This may be tough due to the limited booking availability, so don't worry if you aren't able to do it all before CA1.
- ✓ Knowing all the assessments before CA1 starts and their deadlines (Written clerking + ROCs)

What should you do during CA1?

- ✓ Ask who your CA1 placement tutor in the GP is during your 1st week
- ✓ Know your way around the building and explore the town if you can. I took lots of pictures of Bridgend whenever it was sunny outside.

Having spoken to other more experienced GEM students, I've listed some good suggestions and advice:

- ✓ Introduce yourself to the GP team – not just the doctors, but also the nurses. The nurses are always happy to sign things off for you.
- ✓ Take the opportunity to put yourself out there and don't be afraid! Get stuck in and get as much hands-on experience as you can. The earlier you start, the more experienced you'll become by the time you start FY1.
- ✓ Be curious and ask lots of questions!



Top Tips for Placement

Otis Franklin-Bignall, Year 1 GEM student, shares his Survival Guide to Neath Port Talbot Hospital:

Neath Port Talbot hospital (NPTH) officially opened in 2003, replacing the Neath General Hospital. NPTH is a generalist hospital, which primarily cares for patients with chronic diseases, and those in need of rehabilitation. Despite this, NPTH does have a minor injuries unit, which is run by an incredible nursing staff team.

NPTH has approx. 190 beds and offers a variety of wards/services to its patients. Some of these wards/services include the neurorehabilitation, fracture, anaemia, rheumatology and dermatology units. Orthopaedic elective surgery, geriatric medicine, stroke, and psychiatry wards are also in NPTH too.

Do's:

- Attend Dr. Wafa's teaching sessions! In these sessions course content is expanded upon, through linking conditions to the practical challenges of managing patients from both a clinical and social perspective. (Get your notepads and pens ready for some S-tier notes).
- Ask for help from the team in the education centre! Everybody is incredibly friendly, and if you have any concerns, the team in the education centre are more than willing to help you.
- Make sure you check out the canteen, as the food is in a world of its own, at too good to be true prices (a standard full fry up with a coffee will cost you around £3.50).

Don'ts:

- Feel bad asking to go to other wards if there isn't too much for you to do on a particular day. The tempo in NPTH is a little slower in comparison to Morriston and Singleton hospitals, so you need to be prepared to use your initiative and seek out learning opportunities on other wards.
- Leave sign offs (especially cannulation) in the clinical skills lab to the last minute. If you don't get things signed off in the clinical skills lab before placement, you'll limit the amount of hands-on experience you can get on your placement.
- Rock up when you feel like it! As tough as early mornings may be, try your best to get in before 9am, as it gives you the best opportunity to see what learning opportunities are available on your ward. Plus, it gives you the opportunity to quickly grab a 5-star breakfast from the canteen.

General advice:

Make sure you look presentable on placement, as you're representing not just yourself, but also the university on placement too. Furthermore, although it can be a little intimidating asking members of the clinical team questions, the more you ask, the more you learn, plus you feel more integrated into the clinical team, as they appreciate students who are more likely to engage with the tasks on the ward, than those who don't. Most importantly, have fun, and enjoy the company of those in your placement group when you get the opportunity to do so.

Charlie's Personal Guide to Hospital Canteens

Final year GEM student/hospital canteen veteran Charlie Burton offers his thoughts on the state of the canteens across Wales:

"Whenever I get gloomy with the state of the world, I think about the arrivals gate at Heathrow Airport. General opinion's starting to make out that we live in a world of hatred and greed, but I don't see that. It seems to me that love is everywhere." While mulling this piece over this December with *Love Actually* in the background, it got me thinking, that for me, this place of love and joy is the hospital canteen. Not just because we spend much more time in canteens than airport arrivals halls, but because we can all relate to the scenes of happy porters, doctors, nurses or phlebotomists scoffing and laughing over their Friday fish dinner.

Much like the new Foundation School Programme allocation system, there will be no ranking of canteens today. Instead (unlike said new allocation system), we will focus on the positives, the small unique elements that make each canteen just that - unique. While it would be great to mention all canteens, we only have time for the highlights reel.



Swansea Bay has much to offer when it comes to cafeteria facilities. The golden egg is really **Morriston Hospital** with an excellent array of freshly made paninis and sandwiches in the side cafe, but most importantly Julie's trifle. A piece of sheer culinary perfection, substituting the sponge layer for strawberry angel delight and scattering small emerald like pieces of mint aero over the top. Julie you are a legend.

Neath Port Talbot Hospital performs well with its airy atrium, although please please give us back a daily hot dessert with custard. An honorary mention to the smallest canteen in this journalist's travels, which is **Cefn Coed**. Compact but serving hot jacket potatoes and a range of cold snacks - a must visit for any keen canteen connoisseurs.

The jewel in the Wales cafeteria crown though must go to **Ysbyty Gwynedd** in Bangor. A fantastic array of hot and cold meals, including much needed dessert. Plus, absolute doorstops of homemade flapjacks that many a Swansea student have been seen rooting through.

As this canteen columnist has not himself done the full traverse of Swansea University Placement canteens, I have relied on trusted roving reporters for some of the more remote canteens.

Canteen regular Evan Kneath had this to say about **Glangwili Hospital**: "Hands down the best canteen I've ever been to. Breakfast baps on soft flour rolls and they cut the sausages in half for you, pre-buttered. Authentic thick cut bacon. So good." If that doesn't make you put in a request for Hywel Dda then I don't know what will. Thanks Evan.

Sadly that is all we have time for on my favourite topic of canteens. But perhaps just ask yourself this, while the canteens of Brisbane, Melbourne and Sydney might have sushi counters and organic salad bars, will they have that love, that feeling of a *cwtch* that a Cymru canteen has imparted on us at all at one time or another? This reporter thinks not.

Critical and Clinical Thinking: Bias in Medicine

Year 2 GEM student Jonson Foo reflects on the role of bias in medicine and our collective responsibility in addressing this:

As medical students we have all heard of bias, discrimination, and prejudice. We attend the professionalism lectures (for the most part). We nod, agree, and acknowledge that treating people poorly based on race/ethnicity/sex/gender expression/religion/sexual orientation/social class/disability is bad. We take it on board, and we put it aside. Because surely, "I've never done or said anything bad against [enter group of marginalised people] and I wouldn't as a doctor".

But as we move into a more socially conscious world, we need to register that bias and discrimination aren't just pitchforks and torches, or slurs and aggression. It can be subtle, it can be systematic and it can be seen in medicine.

Discrimination isn't always explicit, and it's not always intentional. Take Dr Takuo Aoyagi, a brilliant man to whom we can attribute an insurmountable amount of saved lives. Because he invented something so intrinsic to medicine that everyone from healthcare assistant to consultant relies on. The Pulse Oximeter¹.

A bit of context and explanation on how the oximeter works (don't worry this isn't on the exams, as far as I know [but please double check the curriculum I don't want to be sued]).

1. The oximeter clips onto your fingertip.
2. It emits red and infrared light through your finger.
3. The light passes through your skin, tissue, and blood.
4. A sensor on the other side of your finger measures how much light passes through.
5. The device calculates the percentage of oxygen in your blood by seeing how much light has been absorbed by oxygenated red blood cells

Long story short: more light absorption = more oxygen²

But you know what else absorbs light really well? Melanin³. Dr Aoyagi invented the oximeter in Japan, which is for the majority, a very homogenous population in terms of skin tone¹. Therefore, people with darker skin tones weren't really factored into the invention. Studies show that that the oximeter says people with darker skin have 1% - 22.1% more oxygen than they actually do, especially in severe hypoxia⁴. For relevance, please consult your nearest NEWS chart (this is a bit more examinable). Even 1% spO₂ can be the deciding factor whether a patient gets escalated, especially when patients' NEWS scores are on the borderline.

¹ Miyasaka, K., Shelley, K., Takahashi, S., Kubota, H., Ito, K., Yoshiya, I., ... & Gawande, A. (2021). Tribute to Dr. Takuo Aoyagi, inventor of pulse oximetry. *Journal of anesthesia*, 35, 671-709.

² Chan, E. D., Chan, M. M., & Chan, M. M. (2013). Pulse oximetry: understanding its basic principles facilitates appreciation of its limitations. *Respiratory medicine*, 107(6), 789-799.

³ Rice, J., Dixon, J., Renfrew, E., Nemeth, D., Finkelstein, A., Gernhofer, Y., ... & Mifflin, S. (2024). Impact of epidermal melanin on peripheral pulse oximetry measurements. *medRxiv*, 2024-09.

⁴ Gudelunas, M. K., Lipnick, M., Hendrickson, C., Vanderburg, S., Okunlola, B., Auchus, I., ... & Bickler, P. E. (2022). Low perfusion and missed diagnosis of hypoxemia by pulse oximetry in darkly pigmented skin: a prospective study. *Anesthesia & Analgesia*, 10-1213.

Critical and Clinical Thinking: Bias in Medicine

And what big bad wolf respiratory infection was the oximeter crucial in?

COVID (sorry to bring it up, but it was a very stressfully enlightening time. "Through the flames of hell something can be forged"⁵ or something like that). People of colour, especially Black and Asian Minority Ethnic groups (BAME) suffered inequitably more, and were more likely to die from COVID⁶.

Bias (systematic and unintentional): oximeters overestimate oxygen levels in darker skin especially in respiratory diseases

Consequence (systematic and unintentional): BAME and people of colour were more likely to suffer with COVID, less likely to be appropriately escalated, and more likely to die⁷

This is not to demonise Dr Aoyagi or undermine his work. Pulse Oximeters are still an essential part of treatment, and still save people's lives. So please don't treat oximeters and spO2 readings like my last minute OSCE revision notes and throw them in the bin. Please use the oximeter, NEWS chart, and spO2 readings in accordance to your local NHS trust guidelines (again I don't want to be sued).

It just shows that as doctors we might have the best of intentions, but we might still have biases that we aren't aware of. In pressured and demanding situations, numbers like NEWS charts become a quick way of prioritising patients. But we treat the patient not the numbers, if the patient is visibly suffering and might have a problem which will affect their O2 and their NEWS is normal, use your better judgement.

So please use your clinical judgement and critical thinking when you see that a particular health problem affects a marginalised group. Quite often this is brushed off as an inherent problem for that group "group X is more at risk of Y because that's the way it is", but often this can be due to discrimination and/or social inequalities which we need to highlight and solve. And if it is due to biological reason, it's our job to find out why and how to fix this. I.e., Calcium Channel Blockers are better than ACEi and ARBs in treating hypertension in Black people (this is very examinable you're welcome)⁸.

But Jonson, what is the solution? Assuming you're reading this in a meeting or a lecture. Look to your left, your right, look to a reflective surface. We are the solution. It's up to us to advocate for our patients, friends, family and fellow human, especially when patterns of discrimination appear. Ask yourself why this particular group, ask them how they are, and then ask what can you do for them. Read more into social and health issues faced by marginalised groups of people. No matter who you are, you have a voice, and you can make space for those people who need to be heard.

If you have any comments on this issue please write in.

⁵ Credit to myself, unless you find out someone else originated that, then credit to them.

⁶ Martin, D., Johns, C., Sorrell, L., Healy, E., Phull, M., Olusanya, S., ... & Fabes, J. (2024). Effect of skin tone on the accuracy of the estimation of arterial oxygen saturation by pulse oximetry: a systematic review. *British Journal of Anaesthesia*.

⁷ Phiri, P., Delanerolle, G., Al-Sudani, A., & Rathod, S. (2021). COVID-19 and black, Asian, and minority ethnic communities: a complex relationship without just cause. *JMIR public health and surveillance*, 7(2), e22581.

⁸ Gopal, D. P., Okoli, G. N., & Rao, M. (2022). Re-thinking the inclusion of race in British hypertension guidance. *Journal of Human Hypertension*, 36(3), 333-335.

'My Three Words' Student Project

Julia McLaughlin, Clinical Teaching Fellow at Withybush Hospital, introduces the 'My Three Words' project:

The 'My Three Words' Project has successfully run for the second year! Utilising the 'What Three Words' app to assign CA1 students to a bed space to follow throughout their acute placements in Withybush Hospital. This project culminated in students creating and displaying fantastic multimedia work for their ward teams to reflect on with them. Please see an example of one student's art and reflection from the project.

Tiffany Edmonds, Year 1 GEM Student:

This piece was created using materials found within the clinical setting. It was drawn over a continuation history sheet, using pens from the ward, and smudged with water from the taps to represent how personal experiences are interwoven with the clinical environment. The digital version is overlaid with more colour and detail, using an iPad. This iPad was also used on the ward. The sketch depicts moments from an arterial blood gas (ABG) test, seen through the patient's eyes—something routine for a healthcare worker, but often significant and uncomfortable for the person receiving it. For me, it captures the moment before I punctured someone with a needle for the first time and the apprehension I felt in that instant.

The patient was an elderly man with heart failure, admitted for worsening breathlessness. Having been in and out of the hospital many times, the beeping monitors and hushed voices were all too familiar to him. He spoke of his frustration with being admitted once again, and yet when I explained the ABG, he simply sighed and accepted it as necessary. And when I performed the procedure, he barely flinched.

My sketch aims to capture this moment: the gloved hands feeling for his pulse, the tension in his wrist as he braced himself, the needle poised above the skin.

The "what3words" defining this location—'Doctor, Usage, Wiping'—were supposedly random, a digital coordinate system marking a hospital bed, yet they felt extremely relevant. 'Doctor' represents not just my presence as a medical student, but also the quiet responsibility that came with performing this procedure—the first step in a career that will include countless minor procedures like this, and possibly far more invasive ones. 'Usage' reflects the clinical nature of medicine, the reliance on tools and techniques, and the precision required to get it right. And 'Wiping' is symbolic of the entire process - the emphasis on hygiene, the act of cleaning the patient, the equipment, and ourselves to ensure a smooth, controlled process, ready for the next patient.

